

In New Hampshire, the Medicaid population is enrolled in a fee-for-service plan without assigned primary care physicians (PCPs) authorizing referrals to further care. Children in SCHIP are enrolled in a Health Maintenance Organization (HMO) product, currently managed by Anthem, that includes traditional HMO elements like PCPs. The population represented in the CHIS commercial data is a mixture of Preferred Provider Organizations (26%), HMO (52%), Point-of-Service (12%), and Indemnity (10%).

Interpretation of Results and Limitations

This is a study of children covered by three different types of health plans (Medicaid, SCHIP, and NH CHIS commercial) conducted in New Hampshire. The large number of covered members studied lends credibility to the findings. However, a number of cautions about the data used and results of this study are provided. This study was based on administrative eligibility and claims data. Differences in provider or insurer claims coding, data processing, or reimbursement arrangements may contribute to the variances shown in this report. Differences in benefit packages and coding by NH CHIS commercial insurer products (Preferred Provider Organizations (PPO), HMO, Point-of-Service, Indemnity or Third Party Administrator (TPA)) may also contribute to variances shown in this report. Because of potential for negative bias (reduced rates) in the NH CHIS commercial insurance estimates, children enrolled in Indemnity and TPA plans (13% of children in the NH CHIS commercial data) were excluded from the claims-based HEDIS measures reported. Children enrolled in NH CHIS commercial Indemnity and TPA plans were included in all non-HEDIS sections of the report. The New Hampshire CHIS commercial population contains information on those residents whose claims are included in the NH CHIS database, which generally only includes members whose policies were purchased in New Hampshire. Areas close to the borders of New Hampshire may be less well represented than areas in the interior. Additionally, companies that self-fund their health care and do not use a TPA to pay claims are not captured in the data set. Because of these two factors, this report underestimates the number of children covered by NH CHIS commercial insurance in New Hampshire.*

While it may be of interest to evaluate children who migrate between the Medicaid, SCHIP, and NH CHIS commercial insurance plan types, there were limitations in the ability to track children who changed insurance plans or insurance plan types during the year. A NH CHIS study was completed in 2009 to track migration between plan types during a three year period, especially with regard to disenrollment and reenrollment in Medicaid and provides insight into patterns of enrollment and disenrollment.³¹

This study compared insured populations that were very different from each other. Previous NH CHIS annual reports on children were limited in the evaluation of health status. This report provides a more detailed evaluation of health status by using clinical risk grouping (CRG). Utilization and payment rates in this report are standardized for population differences in health status and age were added for this SFY2008 version of the annual report on children's health insurance and have been updated with SFY2009 data for this report. *

The statute requiring submission of data is limited to areas regulated by the NH Department of Insurance. Children's Health Insurance Programs in New Hampshire, SFY2009 Office of Medicaid Business and Policy, NH Department of Health and Human Services, October 2010 4